Reg. No. RMEE2552994
PURE PATH LAB
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Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779

Mrs RANI DWIVEDI

Age / Sex : 42 years / Female
Reg. no. : PPL-20261016-02645

PPL-20261016-02645
Registered on : 16/10/2026 12:42 PM
Collected on : 16/10/2026 12:14 PM
Received on : 16/10/2026 12:19 PM
Reported on : 16/10/2026 02:15 PM

HAEMATOLOGY
COMPLETE BLOOD COUNT (CBC)

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN		14.0	g/dl	12 - 15
TOTAL LEUKOCYTE COUNT		5,800	cumm	4800 - 10800
DIFFERENTIAL LEUCOCYTE COUNT				
NEUTROPHILS		48	%	40 - 80
LYMPHOCYTE	**H**	**45**	**%**	**20 - 40**
EOSINOPHILS		02	%	1 - 6
MONOCYTES		05	%	2 - 10
BASOPHILS		00	%	< 2
PLATELET COUNT	**L**	**1.00**	**lakhs/cumm**	**1.5 - 4.1**
TOTAL RBC COUNT	**L**	**3.89**	**million/cumm**	**3.9 - 4.8**
HEMATOCRIT VALUE, HCT	**L**	**35.9**	**%**	**36 - 46**
MEAN CORPUSCULAR VOLUME, MCV		92.3	fL	83 - 101
MEAN CELL HAEMOGLOBIN, MCH	**H**	**36**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**39**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV	**H**	**14.2**	**fL**	**6.5 - 12**
P-LCR	**H**	**56.3**	**-**	**19.7 - 42.4**
R.D.W. - CV		12.8	%	11.6 - 14
P.D.W.	**H**	**17.0**	**fL**	**9.6 - 15.2**
R.D.W. - SD		40.6	fL	39 - 46

PURE PATH LAB
Palce of Accuracy & Your Satisfaction

Fully Computerized Lab Equipped with modern technologies
Same day Reporting of Thyroid Profile, FSH, LH, Prolactin, AMH, Testosterone, HCG, Ferritin, PSA
Hepatitis, TB, PCR, Cancer, & Cardiae Markers Etc.
Clinical Correlation is essential for final diagnosis in case of disparity test must be repeated
This report is not valid for medicolegel purpose

Possible causes of abnormal parameters

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

BIOCHEMISTRY
BLOOD SUGAR FASTING & PP

TEST		VALUE	UNIT	REFERENCE
FASTING BLOOD SUGAR	**H**	**175.9**	**mg/dl**	**70 - 100**
BLOOD SUGAR PP	**H**	**206.2**	**mg/dl**	**< 180 mg/dl**

Clinical Notes

Elevated glucose levels (hyperglycemia) are most often encountered clinically in the setting of diabetes mellitus, but they may also occur with pancreatic neoplasms, hyperthyroidism, and adrenocortical dysfunction. Decreased glucose levels (hypoglycemia) may result from endogenous or exogenous insulin excess, prolonged starvation, or liver disease.

Fasting Glucose	2 hours PP Glucose	Diagnosis
<100	<140	Normal
100 to 125	140 to 199	Pre Diabetes
>126	>200	Diabetes

A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes.

~~~ End of report ~~~

Scan QR Code to Book other Investigations , Thank You for Visit.

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